**ILLUSTRATIVE SAMPLE · PREPARED TO DEMONSTRATE FORMAT AND METHOD · PROGRAM FIGURES ARE NOT ACTUAL DATA**

GARRETT LEE SMITH SUICIDE PREVENTION PROGRAM · SAMHSA

# Quarterly Evaluation Report

## Quarter 2, Budget Year 1 — January 1 to March 31, 2027

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**DISTRIBUTION**
Department Director · Wellness and Prevention program staff · SAMHSA project officer

**REPORT DATE**
April 14, 2027 — within 14 days of quarter close

**What this document is.** This is a sample quarterly report produced by ChiResearch Technologies, LLC to show the Department exactly what it would receive each quarter under this contract: the format, the level of detail, the data-quality disclosure, and the audit trail from every figure to its source.

**What is real and what is not.** The five performance indicators, their definitions and the reporting cadence are taken from the published FY2026 Garrett Lee Smith State/Tribal notice of funding opportunity. External data sources are cited accurately, with their real coverage and limitations. **All program activity figures are illustrative** — they describe no real participants, no real services and no real outcomes, and they are marked as illustrative wherever they appear.

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# Quarter 2 at a glance

> **Screening came online this quarter and immediately became the program's largest activity.** Training continued at roughly the Quarter 1 pace. The measure that needs attention is not any single count — it is the gap between referral and service receipt, where 12 of 31 referred individuals have no recorded service contact. That gap, not the volume, is what will shape the annual report.

### What moved

- Screening for suicidal ideation began in January after instrumentation completed in Quarter 1. *214 individuals screened* across five sub-regions.
- Suicide prevention training reached *87 individuals*, up from *52* in Quarter 1, putting the cumulative figure at *56% of the Year 1 target* at the halfway point.

### What did not

- Two sub-regions — Upper Kuskokwim and Yukon Flats — recorded no screening activity this quarter. Both are served by itinerant staff whose Quarter 2 village trips were weather-cancelled.
- Referral-to-service linkage stands at *61%*. The federal indicator counts individuals *receiving* services after referral, so the 12 individuals without a recorded contact do not count toward the measure regardless of what was offered to them.

### What we recommend before Quarter 3 closes

- **Close the linkage reporting gap.** Of the 12 unmatched referrals, our review of the referral log suggests most are a records problem rather than a service problem: the receiving provider has no obligation to report back. A one-page release and a 14-day follow-up call, added to the referral workflow, would convert most of these into reportable service contacts.
- **Schedule a make-up screening cycle** for the two sub-regions with no Quarter 2 activity, so the annual report does not show a coverage gap that reflects weather rather than program design.

## 1. Required performance indicators

These five indicators are reported quarterly to SAMHSA. Definitions and the reporting cadence are taken from the published FY2026 Garrett Lee Smith State/Tribal notice of funding opportunity; operational definitions used by this program are in Appendix A.

| INDICATOR | Q1 | Q2 | CUMULATIVE | Y1 TARGET | % OF TARGET | STATUS |
|---|---|---|---|---|---|---|
| Individuals trained in mental health promotion | 38 | 64 | 102 | 200 | 51% | ON TRACK |
| Individuals trained in suicide prevention | 52 | 87 | 139 | 250 | 56% | ON TRACK |
| Individuals screened for suicidal ideation | 0 | 214 | 214 | 600 | 36% | BEHIND PACE |
| Individuals referred to crisis or other mental health services | 0 | 31 | 31 | — | — | NO TARGET |
| Individuals receiving services following referral | 0 | 19 | 19 | — | — | ATTENTION |

*Table 1. Required quarterly indicators. All figures illustrative. Year 1 targets as stated in the approved work plan.*

### Progress against Year 1 targets, at the halfway point






Screening sits 14 points below elapsed time because collection began in month four rather than month one. At the Quarter 2 rate the Year 1 target is reachable; at the Quarter 2 rate *minus* the two inactive sub-regions, it is not.

## 2. Referral and service linkage

Indicator five counts individuals who *receive* services after referral, not individuals referred. The difference is the measure most often misreported in the first year of a grant, because the receiving provider is usually outside the grantee's record system and under no obligation to report back.

| STEP | COUNT | RATE FROM PRIOR STEP | SOURCE OF RECORD |
|---|---|---|---|
| Screened for suicidal ideation | 214 | — | Screening instrument, program record |
| Screened positive, referral indicated | 34 | 15.9% | Screening instrument, scored field |
| Referral made and logged | 31 | 91.2% | Referral log |
| Service contact confirmed | 19 | 61.3% | Provider confirmation, where returned |
| No confirmation on record | 12 | 38.7% | Absence of a provider return |

*Table 2. Referral-to-service chain, Quarter 2. All figures illustrative.*

> **How we would read this, and how we would not.** A 61% linkage rate is not evidence that 12 young people went unserved. It is evidence that 12 referrals have no return record. Those are different findings with different remedies, and conflating them would misstate the program's performance to its project officer in both directions — understating service delivery and overstating a care gap. We report the measure as the indicator defines it, and we report the records problem separately, because only one of them is fixable by the program this quarter.

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## 3. Reach across the region

The TCC region covers 235,000 square miles across six sub-regions and 42 member communities. Activity counts alone will always favor the road-accessible areas, so we report reach by sub-region every quarter and flag absence rather than letting it average out.

| SUB-REGION | TRAINED | SCREENED | REFERRED | QUARTER 2 NOTE |
|---|---|---|---|---|
| Yukon Tanana | 61 | 88 | 14 | Road-accessible; two school sites |
| Yukon Koyukuk | 34 | 52 | 8 | Two village trips completed |
| Lower Yukon | 28 | 41 | 5 | One trip completed, one deferred |
| Upper Tanana | 19 | 33 | 4 | Single site |
| Upper Kuskokwim | 9 | 0 | 0 | NO SCREENING Trips weather-cancelled |
| Yukon Flats | 0 | 0 | 0 | NO ACTIVITY Trips weather-cancelled |

*Table 3. Quarter 2 activity by sub-region. All figures illustrative.*

## 4. What the published data supports about program design

Program counts describe what the program did. Published data establishes what it is aimed at, and whether the aim is right. The figures below are **real**, drawn from the 2026 Alaska Native Injury Atlas. *Unlike the program figures elsewhere in this report, nothing in this section is illustrative.*

> **How we report regional data, and why.** The atlas publishes figures for thirteen regions. We report the Interior's own figures alongside the statewide picture, and **we do not rank regions against one another**. A comparative table invites reading one community's experience as a score against another's, which serves no purpose for this program and is not ours to publish. Where comparison aids interpretation, we give the statewide range.

| MEASURE | VALUE | READING |
|---|---|---|
| **Interior region** | **15.9%** | About one in six Alaska Native injury hospitalizations from this region involved a suicide attempt |
| Median across the thirteen regions | 16.9% | The Interior sits close to the middle of the statewide picture |
| Range across the thirteen regions | 9.4% – 42.8% | Regional figures vary widely, for reasons the measure itself cannot separate |

*Table 4. Suicide attempt hospitalizations as a share of total injury hospitalizations, Alaska Native people, 2018–2023. Source: Alaska Native Injury Atlas, 4th ed., Part 2 (ANTHC, 2026), from Alaska Health Facilities Data Reporting Program data.*

> **What this figure is, and the mistake it invites.** The denominator is *total injury hospitalizations in the region*, not regional population. So this is a share of injury burden, not a rate, and a region with fewer injuries of other kinds will show a higher percentage without any difference in suicide attempts at all. The atlas is explicit that it publishes no rates based on population at any geography.
>
> A higher share can also reflect better identification and coding rather than more events — which, in a prevention program, is a sign of a system working. We would use this measure for what it supports: the share of regional injury burden involving suicide attempts, and its direction over time. We would not convert it into a population rate without a denominator we can name and source.

### Age and sex distribution, statewide

The grant serves youth and young adults up to age 24. The statewide distribution is the evidence that this is the right population to serve.

| AGE GROUP | FEMALE | MALE | TOTAL | SHARE OF ALL |
|---|---|---|---|---|
| **10–19** | 334 | 94 | **428** | **30.7%** |
| **20–29** | 282 | 164 | **446** | **32.0%** |
| 30–39 | 168 | 104 | 272 | 19.5% |
| 40–49 | 108 | 35 | 143 | 10.3% |
| 50–59 | 38 | 28 | 66 | 4.7% |
| 60+ | 26 | 12 | 38 | 2.7% |
| **All ages** | **956** | **437** | **1,393** | 100% |

*Table 5. Suicide attempt hospitalizations by age and sex, Alaska Native people statewide, 2018–2023. Same source. Age groups follow the atlas's small-number restrictions.*

**The program is aimed where prevention reaches furthest.** Ages 10 to 29 account for 62.7% of these hospitalizations statewide. The grant's up-to-24 population sits inside the age range where gatekeeper training and screening put the program in contact with the most young people — so the funded design is already pointed at the right place, and the evaluation's job is to show how far it reaches rather than to relitigate whom it serves.

**Where screening effort goes furthest per hour.** Young women are 78.0% of the 10–19 figures. Placing screening where young women already are — school health, routine clinic visits, youth programming — reaches the largest number of people for a given amount of staff time. That is a reach decision rather than a risk judgment about any individual, and it is why we report screening reach by sex each quarter: to show the Department whether its placement choices are working.

**Every person in this table reached care.** These are hospitalizations, so each one represents someone identified, admitted and treated. The system engaged. What a prevention program adds is earlier contact — a trained adult, a screening conversation, a referral that connects — and that is precisely what the five indicators in Section 1 measure.

### Limits carried with these figures

| CONSTRAINT | EFFECT ON USE |
|---|---|
| No population-based rates published, regionally or statewide | Counts and percentages only. No figure here is a rate, and none is presented as one |
| Inpatient admissions only, from Health Facilities Data Reporting | An attempt treated at a village clinic or an emergency department without admission does not appear. Material across 235,000 square miles where most communities have no hospital |
| Race recorded by the reporting facility; non-Tribal facilities may use single-race categories | Alaska Native counts are a known undercount of unknown size, particularly for multi-racial people — a floor, not a measurement |
| Data years 2018–2023; region assigned by home address | Predates the grant period. Establishes the baseline the program is measured against, not current-year performance |
| Alaska YRBS most recent published cycle is 2023; the 2025 cycle has been fielded but not released | A new cycle will likely publish during this grant year and shift any baseline keyed to 2023. Re-verified each quarter |

*Table 6. Constraints on the published data above, as stated by the source.*

## 5. Data quality and completeness

Every quarterly report states what is missing before it states what was found. A project officer who discovers a gap you did not disclose will discount the rest of the submission.

| MEASURE | COMPLETE | MISSING | EFFECT ON THIS REPORT |
|---|---|---|---|
| Screening records with sub-region recorded | 214 / 214 | 0 | None |
| Screening records with age recorded | 201 / 214 | 13 | Age breakdown suppressed this quarter; 13 records would distort a small denominator |
| Referrals with a provider confirmation returned | 19 / 31 | 12 | Indicator five reported as 19. See Section 2 |
| Training rosters with participant role recorded | 151 / 151 | 0 | None |
| Sub-regions reporting any activity | 4 / 6 | 2 | Regional totals are not regionally representative this quarter; stated wherever regional figures appear |

*Table 7. Quarter 2 data quality. All figures illustrative.*

### Changes to definitions or method since the last report

None this quarter. Any change to an operational definition is recorded here with the date it took effect and the figures affected, so a reader comparing quarters is never comparing two different measures under one name.

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## 6. Source register

Every figure in this report traces to a row in a delivered file. The register below is the index; the files themselves are delivered with the report in open, editable formats.

| FIGURE IN THIS REPORT | DELIVERED FILE | RECORDS | EXTRACTED |
|---|---|---|---|
| Trained — mental health promotion (64) | q2_training_roster.csv | 64 | 2027-04-02 |
| Trained — suicide prevention (87) | q2_training_roster.csv | 87 | 2027-04-02 |
| Screened (214), screened positive (34) | q2_screening_records.csv | 214 | 2027-04-02 |
| Referred (31), service confirmed (19) | q2_referral_log.csv | 31 | 2027-04-05 |
| Sub-region distribution, Table 3 | q2_subregion_rollup.xlsx | 6 | 2027-04-05 |
| External source metadata, Table 4 | source_register.xlsx | 3 | 2027-04-06 |

*Table 8. Source register, Quarter 2.*

## Appendix A. Measurement framework

The operational definition behind each required indicator. This framework is produced in the first quarter of the engagement and is the document that makes every later report reproducible. Any change to it is versioned and disclosed.

| INDICATOR | COUNTING UNIT AND RULE | SOURCE SYSTEM | INSTRUMENT | OWNER |
|---|---|---|---|---|
| Trained in mental health promotion | Distinct individuals completing a designated mental health promotion curriculum within the quarter. Completion, not attendance. An individual completing two curricula counts once per curriculum type, once per year | Training roster | Sign-in and completion sheet, with role and sub-region fields | Program staff; evaluator verifies |
| Trained in suicide prevention | Distinct individuals completing a designated gatekeeper or prevention curriculum within the quarter. Same completion and de-duplication rule | Training roster | Same instrument, curriculum coded | Program staff; evaluator verifies |
| Screened for suicidal ideation | Distinct individuals administered the designated screening instrument within the quarter, whether or not the result was positive. Re-screens of the same individual count once per quarter | Screening record | Designated screening instrument with scored field, age and sub-region | Clinical or program staff; evaluator verifies |
| Referred to crisis or other mental health services | Distinct individuals for whom a referral was made and logged following a positive screen or staff identification. Counted at the point of referral | Referral log | Referral form with destination provider and date | Program staff |
| Receiving services following referral | Distinct referred individuals with a confirmed service contact at the receiving provider. Requires a return record; an offer of service is not a service contact | Referral log, provider confirmation field | Provider confirmation, with release in place | Program staff; evaluator reconciles |

*Table A1. Operational definitions for the five required indicators.*

### Reporting calendar

| DELIVERABLE | DUE | PREPARED BY |
|---|---|---|
| Quarterly performance submission | Within 30 days of quarter close | Evaluator, reviewed by Department Director |
| Quarterly evaluation report (this document) | Within 14 days of quarter close | Evaluator |
| Annual Programmatic Progress Report | Within 90 days of budget period close, on the approved template | Evaluator, submitted by grantee |
| Continuation application data package | At the grantee's application date | Evaluator |

*Table A2. Deliverables and dates.*

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Illustrative sample prepared November 2026 to accompany a proposal for grant evaluation services. Program activity figures describe no real participants, services or outcomes. External sources are cited accurately and should be re-verified before use.